

A narrative inquiry approach to understanding senior intensive care nurses' experiences of working with new graduate nurses

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Abstract

Aims and objectives: To report a qualitative inquiry exploring senior registered nurses' stories of experience working with new graduate nurses in the intensive care unit.

Background: While new graduate nurses' perceptions of transitioning into professional practice in the intensive care unit have been examined, few studies have explored the experiences of the senior registered nurses working alongside them.

Design: A narrative inquiry methodology informed by Dewey's (1938) theory of experience.

Methods: Individual in-depth, unstructured, interactive interviews were conducted with five senior registered nurses. The inquirer co-composed individual narrative accounts with each participant. Narrative inquiry's three commonplaces of temporality, sociality and place formed the lens of analysis.

Thematic analysis identified two overarching threads that resonated across the narrative accounts. This inquiry adheres to COREQ checklist reporting.

Results: The overarching threads 'Reverberations' and 'Caring' depict the experiences of senior registered nurses. 'It's Dangerous', 'Patrolling Like Surf Lifesavers', 'We Carry Them', 'Survival Mode' and 'Enjoyable Moments' are minor threads describing the challenges, while 'I've Been There', 'They Must Ask Questions' and 'Not In My Backyard' reveal their insights.

Conclusions: Working with new graduate nurses increased senior registered nurses' patient surveillance and workload, contributing to stress, pressure and feeling overwhelmed. However, senior registered nurses felt an obligation to care for new graduate nurses, who they perceived as still acquiring the skills to autonomously care for critically ill patients. Therefore, senior registered nurses require support themselves.

Relevance to clinical practice: Senior registered nurses perceive workplace constraints as impeding their capacity to provide sufficient support and surveillance to new graduate nurses. The gap between new graduate nurses' clinical capacity and patients' needs underpins senior registered nurses' recommendation that new graduate nurses should not be placed in intensive care units unless additional workforce support is provided, safeguarding their development as well as patient safety.

KEYWORDS

critical care, experiences, graduate nurses, intensive care, narrative, nursing education, qualitative approaches, registered nurses, work satisfaction

1 | INTRODUCTION

Senior registered nurses (SRNs) working in the intensive care unit (ICU) have acquired the skills and knowledge to care for the most critically ill patients. Historically, registered nurses (RNs) did not commence their career in critical care areas of a hospital. Rather, they had one year of clinical experience in acute hospital wards which was thought to provide a better foundation to enter the challenging ICU environment. However, in 2009, a selection of new graduate nurses (NGNs) employed by the inquiry site began their career in the ICU (Saghafi et al., 2012). The SRNs bear most of the responsibility for guiding NGNs' development, often in addition to their own clinical responsibilities. As ICUs continue to experience staffing pressure from international nursing shortages (Sawatzky et al., 2015), an inquiry into the working relationship of SRNs towards NGNs was needed.

The global nursing shortage can be explained as a supply and demand problem. Demand for RNs continues to grow amid increasing patient numbers needing hospital-level health care. Contributing factors include an increasing reliance on technologically advanced health care plus the increased age and growth of the population (Marc et al., 2019). Concurrently, a worldwide under-supply and predicated shortage of RNs can be explained by an ageing RN workforce (Marc et al., 2018), a reduction in nursing graduates, NGN attrition from hospital workforces (Hussein et al., 2019) and unhealthy work environments (Ulrich et al., 2014).

Areas such as ICUs may experience greater nursing shortages (Sawatzky et al., 2015) due to onerous workloads, an ageing workforce, attrition from the nursing workforce and high patient acuity (Duffield et al., 2014). Caring for critically ill patients is demanding, both physically and emotionally, evoking a greater propensity to burnout than other clinical areas (American Association of Critical Care Nurses [AACN], 2016).

Nursing workforce attrition negatively affects healthcare efficiency and cost, ultimately reducing the quality of patient health care (Duffield et al., 2014). Nurses' ability to deliver high-quality patient care is affected by hours of practice, nursing skill mix and practice environment. These factors are associated with rates of adverse patient events (Twigg et al., 2013). Those NGNs who are not supported on an evidenced-based Transition to Professional Practice programme, self-report more negative safety practices, errors, decreased overall competence and increased stress (Spector et al., 2015). SRNs are a vital NGN resource, providing clinical improvement opportunities, advice, support and education (Bortolotto, 2015).

Some SRNs may become discouraged and frustrated when faced with the increased workload of repetitive NGN recruitment, orientation and preceptorship cycles (Ballem & MacIntosh, 2014). When SRNs are in short supply or are stressed or over-worked,

What does this paper contribute to the wider global community?

- SRNs describe negative emotions such as frustration, stress and guilt when balancing ICU clinical responsibilities while supporting NGNs. These negative emotions result from the perceived risk to the provision of safe and quality ICU patient care when increased SRN workloads impede their capacity to provide sufficient NGN supervision.
- Conversely, SRNs describe positive feelings such as satisfaction and pride when NGNs flourish. Committed to supporting NGNs' clinical and professional development, SRNs recommend additional resources, time and supernumerary staff to provide adequate patient surveillance, NGN guidance and continuing in situ clinical education.

dissatisfaction, burnout and high turnover can result (AACN, 2016). Baumberger-Henry (2012) reports that some SRNs interact negatively with NGNs. Consequently, NGNs may become disheartened, contributing to higher attrition rates. Reducing both SRN and NGN turnover rates is an important goal to reduce the associated human and financial costs that affect healthcare budgets as well as patient and staff outcomes (Duffield et al., 2014).

2 | BACKGROUND

A systematised review process guided a literature search from January 2000–November 2018, with inclusion criteria of original research studies reporting SRNs' experiences or views of NGNs working in ICU. Research not situated in ICU or specifically related to NGNs' or nursing students' ICU experience was excluded. The literature search process is provided in Box 1.

Although the literature explores NGNs' experiences transitioning to the workplace, including ICU, the search identified no research studies that solely explored SRNs' experiences working in ICU with NGNs. Three studies (Ballem & MacIntosh, 2014; Baumberger-Henry, 2012; O'Kane, 2012) partly addressed inclusion criteria.

Advanced communication, critical thinking and clinical knowledge, together with the capacity to apply these skills in clinical practice, are crucial to nursing (Saghafi et al., 2012). Despite this, NGNs begin their careers without the expertise, experience or confidence to work independently in settings impacted by escalating

BOX 1 Literature Search Process

- Initial search 'terms of 'new graduate nurses,' 'newly qualified nurses,' 'new to practice nurses,' 'expert nurses,' 'senior nurse,' 'critical care nurse' and 'intensive care nurse' were used' (Whittam, 2018, p. 15) in CINAHL with full text and MEDLINE databases.
- The 'reference lists of relevant retrieved research articles were searched to identify other potential studies' (Whittam, 2018, p. 15).
- Due to the limited number of studies revealed, a research librarian guided a repeat CINAHL database search, expanding both major and minor headings to include 'the terms 'intra-professional relations,' 'mentorship,' 'collaboration,' 'negotiation,' 'dissent and dispute,' 'criticism,' 'work experience' and 'job experience' (Whittam, 2018, p. 15).

patient acuity and workload intensity. Studies exploring NGNs' experiences transitioning into professional practice describe theory-practice gap (Gohery & Meaney, 2013), transition shock (Duchscher, 2009), a lack of confidence (Ortiz, 2016), unsupportive work environments (Parker et al., 2014) and horizontal violence (Parker et al., 2014). When they transition directly into ICUs, NGNs experience a spectrum of emotions (Elias & Day, 2020), are challenged when recognising the necessity to possess highly developed communication skills (Saghafi et al., 2012), are scared of making mistakes (St Clair, 2013), are unsure of socialisation processes (Innes & Calleja, 2018) and have a desire to learn (St Clair, 2013).

Descriptions of programmes designed to orientate NGNs to ICU (Bortolotto, 2015; Friedman et al., 2011; Welding, 2011) revealed a focus on NGNs' distinct learning needs, using various educational methods to support them while addressing patient safety, nursing attrition, safeguarding of RN staffing numbers and delivery of high-quality nursing care. However, the programmes did not investigate the SRNs' experience of working with the NGNs.

The studies that partly addressed inclusion criteria described the SRNs' perceptions of NGNs' ICU experiences as the second phase of a study (O'Kane, 2012) or included SRNs from an ICU alongside other hospital wards (Ballem & MacIntosh, 2014; Baumberger-Henry, 2012). Although O'Kane's (2012) study sought to explore the NGNs' ICU working experience, the parallels and disparities between SRNs' and NGNs' perspectives of the phenomena were examined. Despite the SRNs' perception that NGNs managed the demanding aspects of the ICU well, the ICU and SRNs were placed under considerable pressure when supporting NGNs, raising concerns regarding the NGNs' lack of hospital knowledge. The rate of preceptor burnout increased with the number of NGNs employed. The authors determined that NGNs require 8 weeks of supernumerary time and be supported by two nursing preceptors. Additionally, a focus on completing competency-based training was thought to contribute to creating a generation of task-oriented RNs (O'Kane, 2012).

Ballem and MacIntosh's (2014) study sought to understand SRNs' perspectives of working with NGNs in various hospital clinical

settings, including critical care [ICU]. Analysis identified three themes: "New Graduates Are Coming", "Keeping Us on Our Toes" and "Carrying the Load" (p. 379). Despite a considerable workload increase when working with NGNs, SRNs were motivated to refresh and update their skills and knowledge; anticipating NGNs' questions and need for procedural support. Although SRNs voiced verbal support for NGNs, they also expressed annoyance when NGNs did not follow advice.

Baumberger-Henry's (2012) study examined SRNs' perspectives on work relationships with NGNs in emergency and critical care [ICU] units. The study determined the following themes: "Lacking Confidence" and "Gaining Acceptance in the Unit Culture" (p. 301). The SRNs expressed frustration with NGNs' fear, lack of confidence, deficiencies in basic skills, critical thinking and time management. The second theme described SRNs' uncivil behaviour towards NGNs, such as belittling or ignoring NGNs' questions and using patronising language. The SRNs recognised their collective silence legitimised the unsupportive behaviour. Baumberger-Henry (2012) emphasised that NGNs should be encouraged and nurtured to ask question without fear of being perceived as burdensome.

Finding scant literature, the necessity to examine the SRNs' experiences when working with NGNs in the ICU was evident. Analysis of SRNs' experiences may reveal interpersonal, cultural and workplace factors influencing workplace relationships, staff well-being, retention and ultimately patient safety.

3 | AIM

The aim of this article was to report a narrative inquiry that explored SRNs' stories of experiences working with NGNs in the ICU.

4 | DESIGN/METHODOLOGY

Narrative inquiry methodology recognises stories of experience as a source of valuable understanding and knowledge (Clandinin & Connelly, 2000). The term 'narrative' refers to the phenomenon of the experience being studied and the method of inquiry for its study (Clandinin & Connelly, 2000). The three-dimensional space of narrative inquiry is informed by Dewey's (1938) criteria of 'experience—interaction and continuity enacted in situations' (Clandinin, 2013, p. 12). The three commonplaces of narrative inquiry that delineate the inquiry space dimensions are 'temporality, sociality and place' (Clandinin, 2013, p. 39). These are defined in Table 1 and were used to guide the exploration of SRNs' stories of experience. The consolidated criteria for reporting qualitative studies (COREQ) 32-item checklist was used to report this inquiry (File S1).

4.1 | Participants

An idiographic approach was used to purposively enrol five self-selecting participants who were willing to tell their stories of

TABLE 1 Narrative inquiry three-dimensional space.

Temporality	Experiences under examination exist in temporal transition, not one stand-alone moment in time independent from its past and its future influence (Clandinin, 2013). Narrative inquirers consider people, places, things and events under exploration as having a past, present and future.
Sociality	Social and personal conditions of both the inquirer and the inquiry participant effect the inquiry. Personal conditions are 'feelings, hopes, desires, aesthetic reactions and moral dispositions' (Connelly & Clandinin, 2006, p.480). Social conditions include an individual's context, the place, encircling forces and factors, existential conditions and people (Connelly & Clandinin, 2006). The inquirer (SW) accounted for and acknowledged in the inquiry the pre-existing and ongoing professional relationship with participants.
Place	One or a sequence of places is 'the specific concrete, physical, and topological boundaries of place where the inquiry and events take place' (Connelly & Clandinin, 2006, p. 480). Narrative inquirers acknowledge and recognise the impact and qualities of place on the inquiry, as, people, places and stories are intricately linked (Clandinin, 2013).

experience working with NGNs in the ICU. This enabled location of individual participant voices and stories of experience within the three-dimensional inquiry space, while facilitating rigorous analysis of each participant's narrative account. The literature presents minimal research determining qualitative sample sizes, although recommendations exist. For example, when referring to narrative research, Creswell and Poth (2016) recommend selecting and observing one or more individuals who have stories and experiences to share. It has been argued that a sample size of 'one single case can be highly informative and meaningful as demonstrated from management and medical research' (Boddy, 2016, p.426). Focussing on narrative inquiry, Clandinin and Connelly (2000) provide exemplars of inquiries involving one, three and five participants.

The inquiry was conducted at a single site, public quaternary referral hospital in an Australian metropolitan city. Recruitment flyers were placed on ICU notice boards. Criteria for inquiry inclusion included: SRNs employed permanently in the ICU, having worked with a NGN in the immediate three months prior to the inquiry start date and were prepared to provide written, informed consent while observing the inquiry protocol. All participants had a pre-existing professional relationship with the first author (SW), a female clinical nurse educator employed at the inquiry site; participants were aware the inquirer had an interest in the topic with the inquiry being undertaken as part of a higher research degree (SW). All recruited participants predominantly worked in the cardiothoracic ICU pod of a larger ICU service.

An ICU context description and definitions of specific vocabulary used in the inquiry are provided in Table 2. Potential participants interested in the inquiry meeting inclusion criteria were enrolled. No participant was excluded or withdrew from the inquiry. The participants presented demographic homogeneity; being, anglophone, Caucasian and female. The nursing participants had accrued an average 13 years of experience in the inquiry site ICU. Two participants rotated through ICU pre-2009, on a Transition to Professional Practice programme final 4-month placement. One participant commenced their career in ICU post-obtainment of their initial nursing qualification. However, this participant's country of origin nursing higher education programme

and clinical exposure differed considerably from the Australian experience. Two participants gained initial nursing experience in acute care wards before being employed in the ICU.

4.2 | Field Text [Data] collection

The first author (SW) undertook unstructured interactive interviews between October 2016–January 2017, negotiating time and space for both inquirer and participant to tell and hear each other's stories of experience (Clandinin, 2013). Playing an active role, the inquirer commenced with a 'grand tour question' (Spradley, 2016). Focused listening, probes and responding to participant questions were used to gain rich details regarding the phenomenon under inquiry. The interview guide is provided in Table 3.

Interview location, date and time were decided by participants; all chose a private location proximal to the ICU. The interviews (average 41.4 minutes) and inquirer's post-interview verbal reflections to support reflexivity were digitally audio-recorded and independently transcribed. Field Text [data] collection to saturation was not pursued; being incongruent with narrative inquiry methodology and the *temporal* dimension of the underlying philosophy of experience.

4.3 | Ethical considerations

The inquiry was approved by the hospital human research ethics committee and received 'cross-institutional recognition of this approval from the [supervising] university' (Whittam, 2018, p. 48). Confidentiality was protected by de-identification and secure storage of participant records and inquirer's field notes. Pseudonyms were used for participants and persons mentioned in the narrative accounts and when referencing participant quotes.

Participant information was provided, and consent was gained prior to interview. Iterative consent was sought throughout the inquiry. Attentive to the relational stance and the commonplace of

TABLE 2 Inquiry term definitions/description.

Intensive care unit (ICU)	The inquiry site ICU department contains two discrete specialities: cardiothoracic surgery and general/neuro/trauma. Although registered nurses (RNs) are mainly allocated to one speciality with separate nursing rosters, they work throughout the department as dictated by staffing activity and clinical demand. During weekday office hours, two clinical nurse consultants, a nurse educator, two clinical nurse educators (CNE) and two nurse unit managers (NUMs) support the ICU nursing workforce. When the NUMs have designated office time, a team leader (TL) is assigned to manage the clinical area. Customarily, the TL commences the shift without a patient load. However, if an unexpected urgent ICU admission occurs, in a context of lean staffing, the TL would take a patient allocation. The nursing workforce in ICU is not augmented by additional healthcare specialists such as respiratory therapists or dialysis nurses. Commonly, one appropriately qualified RN problem solves and is responsible for all patient care of a critically ill patient, including the invasive and technological devices required to deliver organ support (Chamberlain et al., 2018). For example, permanently employed RNs independently manage patients requiring mechanical ventilation, including assessing patients' ventilation requirements, adjusting ventilator settings and all aspects of maintaining an artificial airway. Other advanced technology overseen by RNs encompass extracorporeal membrane oxygenation (ECMO) and ventricular assist devices (VADs), inhaled nitric oxide and intra-aortic balloon pumps. Additionally, appropriately educated and competent RNs assume nursing positions in emergency situations, such as open-chest resuscitation and ECMO cardiopulmonary resuscitation. After a two-day hospital-wide orientation programme, NGNs allocated to ICU on a Transition to Professional Practice programme undertake a three-day ICU induction. Led by CNEs, the induction comprises an ICU orientation, ICU nursing principles, ICU technical skill stations and socialisation. After induction, NGNs are supernumerary (surplus to staffing requirements), shadowing an experienced RN who assumes full responsibility for their assigned patient. 'The supernumerary period comprises seven 8-hour morning shifts for "first rotation" NGNs and three 8-hour morning shifts for "second rotation" NGNs' (Whittam, 2018, p. 7). Each rotation last six months. A 'first rotation' is the first of two 6-month placements and is the NGNs' first clinical experience as an independent RN. A 'second rotation' NGN has worked in another clinical area for six months before working in ICU. During the supernumerary period, each NGN works alongside an experienced RN, assuming incremental accountability for their assigned patient. Post the supernumerary stage, NGNs are independently assigned an ICU patient. To provide continuing NGN support, preceptors are assigned shift-by-shift to NGNs, for an additional 6 weeks. Through the preceptorship period, in addition to their preceptorship obligations, SRNs are allocated patients. At the conclusion of a 6-month ICU placement, NGNs are expected to safely care for a mechanically ventilated patient.
New graduate nurse (NGN)	NGNs are RNs within their first year of clinical practice, post-graduation.
Resource nurse	The resource nurse supports other RNs with workload, break relief and education for less-experienced nurses. Allocation of a resource nurse is decided by patient acuity and available staffing on a shift-by-shift basis, as an example, when more than one patient requires ECMO. The RN who has been designated as the resource nurse is not assigned a patient allocation and remains available to take an emergency admission; thus, they may cease the resource nurse role at any time during the shift.
Senior Registered Nurse (SRN)	SRNs are RNs working in the ICU with at least five years' clinical nursing experience. All SRN participants in this inquiry were team leaders; four were clinical nurse specialists, and all had post-graduate qualifications.
Transition to Professional Practice Programme	The 12-month Transition to Professional Practice programme for NGNs employed at the inquiry site is guided by a framework integrating clinical rotations, supernumerary time, a preceptorship model, regular appraisals, professional development days, mandatory education sessions and socialisation support.

sociality in narrative inquiry methodology, a reflexive approach permeated the inquiry.

4.4 | Analysis

Transcript accuracy was checked by reading the transcript while concurrently listening to the digital recording; this process facilitated inquirer (SW) immersion in the text. Attentive to the three-dimensional space, transcripts were arranged manually into chronologically and/or thematically coherent stories (Mishler, 1995) with minimal smoothing of participants' quotes. The subsequent reconstructed stories were termed tentative narrative accounts (Clandinin, 2013).

Individual participants received a hard copy of their tentative narrative account, and all participants reflected, commented and negotiated elements of their story with the inquirer (SW). Credibility was established when participants confirmed that their final narrative account accurately represented their story. Each narrative account was left 'whole', respecting and honouring the individual voice and story of experience (Appendix S2). An audit trail was maintained, with field notes, transcripts, research reflections and recordings retained.

The final narrative accounts underwent further analysis. A six-phase thematic analysis method (Braun & Clarke, 2006) guided the inquirer's (SW) active identification, analysis and reporting of 'threads' [themes] that resonated across the narrative accounts (Clandinin, 2013). A semantic and inductive approach to thread identification

'Grand Tour' Question	Can you tell me a story of your experience working with NGN in the ICU?
Probes	What are some of the challenging experiences you have had working with NGNs? What are some of the rewarding experiences you have had working with NGNs? How does working with NGNs affect you personally? How has working with NGNs affected you professionally? What are some of the positive aspects of working with NGNs? What are some of the negative aspects of working with NGNs? If you could change any experience you had working with an NGN, what experience would that be and why would you like to make a change?
Clarification	Clarify any terms or potential misunderstandings with the participant.
Conclusion	Ask participant if they have anything else to contribute? Thank participant for their gift.

TABLE 3 Unstructured Interactive Interview Guide.

was adopted (Braun & Clarke, 2006) with all authors (SW, NT, JP) contributing to thread refinement via Socratic questioning.

5 | RESULTS

Two overarching threads, 'Reverberations' and 'Caring' resonated throughout the narrative accounts. The threads exemplify SRNs' experiences working with NGNs in the ICU. Each main thread contained minor threads adding additional levels of understanding; these are depicted in a network diagram format (Figure 1).

6 | REVERBERATIONS

'Reverberations' have a continuing or prolonged effect. The overarching thread Reverberations represents the ongoing effect on SRNs as they support NGNs' entry to ICU nursing practice. The

SRNs commented that the ICU working environment, the *place* of inquiry, is emotional, demanding and stressful. The NGNs' presence exacerbated this situation, affecting SRNs' capacity to meet team-leading role and patient care requirements.

The SRNs' described the ICU *place* as follows:

The patients are a lot sicker now. And we have heaps more specialised equipment and that now. We can have three or four ECMOs [extra corporeal membrane oxygenation] at once. And patients that normally would've just died on the [operating] table, now they bring them out

[Kylie, Line. 128–130] (Whittam, 2018, p. 71).

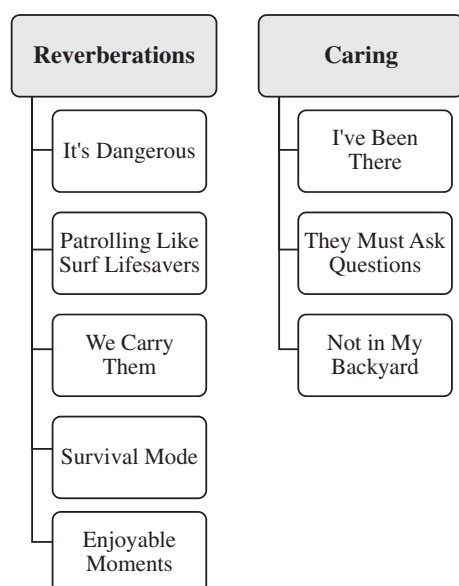
These reverberations had particular repercussions post the NGNs' supernumerary time, when NGNs transitioned from being additional, to being accounted for in the ICU nursing workforce numbers:

As soon then as they have their own patient and you have your own patient, it kind of changes. It becomes a lot more intense or a lot more stressful, I think, probably for both of you because you can't watch what they're doing ... even if you have a kind of quiet patient, you're still busy with your own patient and they're [NGN] a bit lost

[Sarah, Line. 15–19] (Whittam, 2018, p. 71).

The SRNs detailed NGNs' narrow experiences as independent nurses, describing their need for SRN support when delivering basic ICU nursing care and considerable assistance when providing advanced nursing interventions. Therefore, NGNs were perceived to be under-prepared for high-acuity patient care and the unrelenting pace inherent to the ICU:

I think some of them are absolutely stunned. They're like deer in the headlights when they come here. They—it's—it's confronting. If you've never been to ICU, you come here, you hear all the alarms, you see

**FIGURE 1** Thread network diagram.

all the machines and you go like, 'Oh. My. God. What am I supposed to do? Where do I start? I don't know what that machine does. I don't know anything about this?'

[Camilla, Line. 219–223] (Whittam, 2018, p. 72).

The ramifications were further clarified using metaphors of surveillance and juggling.

6.1 | It's Dangerous

The SRNs were cognisant of the mismatch between skills and theory acquired at university and the actuality of ICU patient care demands. They perceived it to be dangerous for NGNs to autonomously care for patients in a high-acuity clinical environment while developing the required level of proficiency.

It's not good for them [NGNs], I don't think, this environment. You just can't afford—I can't afford to trust them, their skill and their knowledge. 'Cause I—you just can't. It's too dangerous

[Kath, Line. 383–385] (Whittam, 2018, p. 73).

When emergencies occurred, SRNs observed NGNs were limited in their capacity to rapidly provide crucial nursing care. Therefore, SRNs immediately intervened to avoid patients being subjected to life-threatening and potentially dangerous complications:

These people [ICU patients] just deteriorate, and you then are putting out a fire. And it's emotional for the new grad [NGN] because often when it gets to that point you—there's only so much talking you can do <laughs>. You've almost got to push them out of the way and just sort the problem out and explain it afterwards. So, it's stressful for you, it's stressful for the patient importantly and it's stressful for the new grad [NGN]. Ah, and that can happen frequently here, unfortunately

[Kath, Line. 323–328] (Whittam, 2018, p. 74).

Oversight by SRNs of the nursing care provided by NGNs was deemed critical to safeguard patient safety and avert potential hospital-acquired complications and associated sequelae:

Well, one new grad on a couple of occasions let the CVC [central venous catheter] disconnect from—without clamping it, so obviously risk of air embolism ... Because, the first time it happened I thought okay, good, she would've learned her lesson now and that's not going to happen again, but then it happened again and then it's just like, oh my god, how—you know?

That's so scary to me ... when you're new, I think it's just so easy to make mistakes

[Sarah, Line. 38–61] (Whittam, 2018, pp. 74–75).

Acute awareness of the negative outcomes that could result from the mix of minimal nursing experience and critical illness led SRNs to feel responsible for safe patient care.

6.2 | Patrolling Like Surf Lifesavers

Attentive to the danger of NGNs working in nursing positions for which they lacked requisite experience and education, SRNs used surveillance processes to protect NGNs' practice and patient safety. However, SRNs considered their surveillance to be insufficient due to workload demands in the high-acuity ICU environment:

You just constantly have to be awake and alert and patrolling. Like, I feel like I'm on ... [the beach] sometimes <laughs>, you're just up and down, up and down, up and down

[Kath, Line. 343–344] (Whittam, 2018, p. 76).

In addition to observational vigilance, SRNs assisted the NGNs with their workload.

6.3 | We Carry Them

The SRNs perceived their concurrent roles in a lean nurse-staffing model with inadequate skill mix as contributors to excessive workloads. Common additional roles performed by SRNs comprise being a resource nurse and/or a code blue team member, educating less-experienced clinicians and precepting NGNs:

You often as a new-in charge find yourself being the—you're actually the resource nurse, so if we don't have enough staff to have two nurses on the ECMO, you are the extra staff on the ECMO, and you cover all the breaks, and you're trying to support the new grads

[Sarah, Line. 151–153] (Whittam, 2018, p. 77).

The SRNs actively advocated for reduced patient risk, requesting managers increase RN numbers so that NGNs were assigned patients within their current level of competence:

We don't have any resource nurses, yeah. And, you really have to fight sometimes to have two nurses on fresh ECMO, and if you have those two nurses then things are unbelievably better, but if you don't have them, you know, the new grads a lot of the time they can just sing [manage on their own] ... You do really

try to get around to them, but I don't know. But like, most of them do very well at the same time. It's just kind of like a bit of an element of an accident waiting to happen

[Sarah, Line. 154–159] (Whittam, 2018, p. 77).

Although SRNs acknowledged that the preceptor position was fundamental to NGNs' development and support, feelings of pressure and responsibility accompanied the role. These feelings were heightened when SRNs were assigned a critically ill patient alongside preceptoring responsibilities:

It's time-consuming preceptoring, making sure that everything is done. It's quite a lot of pressure because you do take responsibility of, you know, what they do. You have to

[Camilla, Line. 23–25] (Whittam, 2018, p. 77).

The SRNs reported self-denial, placing the needs of team members before their own. This facilitated NGNs to take assigned breaks, while preserving acceptable skill mix to safeguard patient safety:

You don't get breaks because you're too worried about the patient's safety. So, you might just ...sit down at the desk and eat your food there, hopefully, or have two minutes—go and heat up for food in the microwave and run back. It's crazy sometimes

[Camilla, Line. 128–131] (Whittam, 2018, p. 78).

Notwithstanding the additional workload related to supporting and supervising NGNs, SRNs urged NGNs to communicate concerns and request guidance; they provided learning opportunities and ameliorated NGNs' workloads:

If they have to take a central line out, they've never taken a central line out. To have that time to show them where they find the ... policy from, do the procedure, like explain the whole thing. It's a lengthy thing to do if you ... look at the policy, explain the procedure, do the procedure together and then have some sort of ... feedback at the end of it ... 'you did great, next time you have to remember to do that, well done

[Camilla, Line. 167–171] (Whittam, 2018, p. 78).

To ensure NGNs were nursing within their scope of practice, SRNs acted as advocates by sourcing suitably skilled RNs to care for high-acuity patients. The extra work involved in negotiating patient allocation changes was construed by SRNs as 'carrying' the NGNs:

You need to replace them with, you know, an agency nurse or someone doing overtime. And it's frustrating and time consuming trying to organise that

[Camilla, Line. 119–121] (Whittam, 2018, p. 79).

Reallocating an NGN to a patient whose care requirements corresponded with the NGN's current skill set was an approach perceived to increase ICU patient safety:

So, I was in-charge, but I had to take a patient because we were too tight across the floor and because ... The only other person that could've doubled say, was a new grad, but I couldn't let her double because it would've been too much for her

[Sarah, Line. 200–202] (Whittam, 2018, p. 79).

The SRNs' workload increased when they provided support to the NGNs.

6.4 | Survival mode

Survival is continuing to live or exist despite difficult circumstances and represents SRNs' ability to work in the particularly stressful ICU setting. When SRNs felt unable to meet the increased workload associated with delivering safe patient care while supporting NGNs, adverse psychological and emotional indicators, such as stress, pressure, frustration and guilt, resulted:

There is just so many things to do. If you want to look after your patient properly and do all the things that you're supposed to do during the day and look after the patient like would be your own family member, to do that detail, then how on earth do you find a couple of hours during that shift to teach the new grad?

[Camilla, l. 182–186] (Whittam, 2018, p. 78).

The SRNs' feelings of accountability to ensure NGNs' potential errors were recognised and remedied before patient injury occurred generated considerable SRN stress. The potential for errors to cause an adverse patient event, or the death of a patient was omnipresent for SRNs:

Terrible! It's a huge responsibility. Absolutely massive
[Camilla, Line. 71] (Whittam, 2018, p.80).

But when it's really busy, then there's the capacity for something to go wrong—and then I'd feel terrible

[Kylie, Line. 222–223] (Whittam, 2018, p. 80).

The SRNs used a juggler metaphor to represent how they managed workload and stress when balancing support of NGNs with care for ICU patients:

Stressfully. I don't know. I don't know how you manage it. Just try the best you can. Just juggle I ... I try to keep as much of an eye as I can on them

[Sarah, Line. 81–83] (Whittam, 2018, p. 80).

During periods of increased patient acuity, when SRNs were balancing concurrent duties or when multiple NGNs were rostered simultaneously on a shift, SRNs were concerned and felt guilty that they were unable to complete tasks to a desired standard while maintaining a high level of NGN supervision. A sense of personal responsibility was assumed by SRNs when colleagues discussed concerns about NGNs' nursing practice:

And you hear things back from other nurses ... They're like, 'By the way, this happened', and ... you just feel guilty that it happened 'cause they're upset, the new grad's upset, you're upset. I'm wondering 'is this my responsibility that the new grad just did that?

[Sarah, Line. 32–35] (Whittam, 2018, p. 80).

The SRNs experienced additional pressure throughout the 'buddy' period when guiding NGNs towards autonomous ICU practice:

You feel under pressure in the first week to give them as much preparation as possible, so they'll be able to take their own patient the following week, but obviously, you can't give them enough, and you can't cover everything that's going to happen and a lot of what you tell them they've forgotten again anyway

[Sarah, Line. 21–24] (Whittam, 2018, p. 81).

Although some SRNs had established methods to create work/life balance, others dreamt or thought about work when at home. The SRNs 'coped' with the demands of ICU work in various ways such as drinking alcohol at home to unwind after a work shift, talking with family members or colleagues and attending counselling to deal with the emotional impact of complex patients:

... burned out sometimes. Oh, it's good, like, I enjoy it, but you're just busy. I take lots of it home. I know I don't—I try not to, like, too much but yeah, you definitely take it home

[Sarah, Line. 370–372] (Whittam, 2018, p. 81).

Although SRNs felt pressure and stress, they also experienced positive rewards from NGNs working in ICU.

6.5 | Enjoyable moments

Educating NGNs stimulated SRNs to update their nursing knowledge and refresh their hands-on skills. Described by SRNs as beneficial for lifelong learning, teaching NGNs resulted in feelings of satisfaction and enjoyment. Questions from NGNs were received positively as they challenged SRNs' practices and thought processes:

Sometimes it keeps your brain going with the questions they ask you. Makes you think about why you're

doing things or that you don't. And then sometimes they might ask me something I don't actually know, so then I have to go look it up. So, in that way it's beneficial

[Kylie, Line. 262–265] (Whittam, 2018, p. 82).

Contrastingly, the SRNs also recognised and appreciated when they learnt from NGNs:

It was a very basic thing that quite a junior person [NGN] taught me something and I was like, 'Wow, that's really awesome to know. I wish <laughs> I'd known that ten years ago

[Kath, Line. 290–292] (Whittam, 2018, p. 83).

Feelings of pride, accomplishment and reward were experienced by SRNs when NGNs progressed in their nursing practice.

It's like a win because when you see a new grad developing, you're proud of them, so it's nice

[Sarah, Line. 211–212] (Whittam, 2018, p. 83).

Sometimes, NGNs exceeded expectations by demonstrating clinical insight and critical thought beyond normal expectations:

Some of the new grads are brilliant and they ask you a question and they just bowl you off your feet 'cause they're so far ahead of where you think that they should be or—that's really good

[Sarah, Line. 213–215] (Whittam, 2018, p. 83).

The reverberations of increased workload in a high-risk environment experienced by the SRNs were counterbalanced by the satisfaction of being able to safely guide the NGNs and share in their journey of skill development.

7 | CARING

The concept of caring denotes the SRNs' culture of nurturing NGNs, echoing the *sociality* commonplace. While SRNs had ambiguous opinions about the decision to allocate NGNs to ICU as a component of Transition to Professional Practice programme, they felt obligated to provide clinical, professional and psychological support, and participated in NGNs' clinical development. This obligation rested on a recognition of inherent danger in the ICU *place*, NGNs' steep learning curve throughout the six-month ICU placement (*temporality*) and the consequences of potential mistakes as contributing towards NGNs' feelings of stress and feeling overwhelmed.

A commitment to the NGNs' integrating ICU specific skills and knowledge was understood by the SRNs as being essential to safeguard the delivery of patient care. The SRNs created educational opportunities for NGNs:

If I come in and it's reasonably quiet, I will try and give the new grad something challenging so that I can support them and try and teach them something
[Sarah, Line. 166–168] (Whittam, 2018, p. 84).

However, SRNs' demonstration of caring extended beyond education and clinical support to genuine attentiveness to the NGNs as individuals. Generating camaraderie with NGNs, SRNs created relationships that supported NGNs to be transparent about their experiences and emotions:

I think you can try and make them feel comfortable ... You've got to talk to them and communicate with them and, um, kind of get an understanding of what they understand ... Rather than looking at what they can't do, kind of working through what they want to learn
[Lisa, Line. 87–90] (Whittam, 2018, p. 84).

The support offered by the whole ICU team was considered by SRNs as an essential component of caring for NGNs:

Like we always work as a team to try and incorporate the new grads because they aren't stand-alone members of staff on their own. They can't be because they just don't have the experience and it's just not fair to—and that's how they suffer when we're really busy because—because the team then that is trying to support them kind of falls apart a bit because we're too busy
[Sarah, Line. 248–252] (Whittam, 2018, p. 85).

The caring demonstrated by SRNs was echoed in their appreciation of the significance of initial professional experiences. They sought to create worthwhile and positive experiences for NGNs' placed in ICU:

It might affect the rest of their nursing career. It's a pivotal thing, your first experience when you come out, whether you enjoy it
[Kylie, Line. 29–31] (Whittam, 2018, p. 85).

Nurturing of NGNs originated from a blend of SRNs' empathy and obligation to develop nurses new to the profession.

7.1 | I've Been There

Thinking *temporally* and reflecting through the lens of their own experiences when inexperienced in ICU gave SRNs insight into NGNs' experiences, motivating SRNs' to be empathetic and caring towards NGNs:

I feel sorry for them because I was there and I know what it's like
[Kath, Line. 136] (Whittam, 2018, p. 86).

As SRNs were acutely cognisant of the overwhelming and pressured ICU work environment and how individuals coped, they actively sought to minimise unproductive communication styles and negative interactions:

I don't mean to be intimidating to—to people. I don't want people to think that I intimidate them. Because that—you know that doesn't really solve anything. Just makes you look like a scary nurse and I—I don't want that
[Lisa, Line. 131–133] (Whittam, 2018, p. 87).

The SRNs used their past experiences to motivate and guide their interactions with NGNs in the present, developing them for the future.

7.2 | They Must Ask Questions

Linking NGNs' ability, willingness and comfort in asking questions to their capability to deliver timely and safe care to ICU patients, SRNs were unable to sufficiently support NGNs' workload, prevent potential patient error or provide timely education if they were not aware of NGNs' concerns. The NGNs' questions warned SRNs of latent patient safety concerns.

During their daily clinical practice, SRNs reinforced the need for NGNs to ask questions:

And I always say that as well to new grads, you know, 'just ask'. Like, you try and reaffirm it like, 'ask questions, ask anyone. It doesn't matter. Just ask the question rather than doing something dangerous
[Sarah, Line. 86–88] (Whittam, 2018, p. 88).

An increased demand for NGNs to ask questions during hectic shifts was emphasised by SRNs, as their ability to provide surveillance was reduced. Explanations were provided to NGNs regarding the possible consequences of not seeking clarification or asking questions when unsure:

You might lose a life in ICU. That's the reality
[Camilla, Line. 69–70] (Whittam, 2018, p. 89).

The substance of NGNs' questions presented SRNs a window into how NGNs were advancing clinically, in addition to their comprehension of the justification underlying clinical decisions and interventions:

... to know that they're actually thinking about what they're doing ... They've been shown how to do it and are just doing it but, not actually understanding why they're doing the things they're doing.
Processing
[Kylie, Line. 216–219] (Whittam, 2018, p. 89).

The SRNs postulated that NGNs' reluctance at times to ask questions potentially may have been to obscure knowledge deficits, a sign of apprehension of the response, or hesitancy in the presence of patient visitors:

Well, the reality is that...they don't always ask, you know. They're too embarrassed to own up... not knowing something that they think they should know. Um, and you discover later on, when things go wrong, that they had no idea really and they didn't come and ask you [Camilla, Line. 61–64] (Whittam, 2018, p.89).

... if family work out that they're a new grad. Some family can be very funny about that, and that makes things <sighs> then they get all nervous

[Kylie, Line. 267–269] (Whittam, 2018, p. 89).

The SRNs actively sought professional relationships with NGNs which were caring, empathetic and supportive, while encouraging them to ask questions when they felt uncertain:

You, kind of have to tune into them a bit and, and try and connect to them and make them feel comfortable. So that if there is an issue that they don't understand, that they come and ask

[Lisa, Line. 71–72] (Whittam, 2018, p. 89).

A tension existed for SRNs in balancing parallel patient care demands while offering NGNs guidance, answers and rationales. Feeling obligated to ensure NGNs provided excellent patient care, SRNs urged NGNs to raise concerns and ask questions. However, answering questions increased SRNs' workload, resulting in increased stress; this was particularly noticeable when multiple NGNs worked on a shift. The SRNs felt frustrated when NGNs did not ask questions nor incorporate recommendations into their daily practice. Despite question fatigue or frustration, SRNs modified their behaviour by communicating openly and respectfully with NGNs:

There's no point in being degrading or—know what I mean? Like, that's not gonna solve anything

[Lisa, Line. 79–80] (Whittam, 2018, p. 90).

Caring for NGNs' development and the safety of ICU patients required SRNs to balance approachable availability for NGN questions while managing their own increased workload. Hence, SRNs required NGNs to be proactive, asking questions when they self-recognised knowledge or skill gaps.

7.3 | Not in My Backyard

The phrase 'not in my backyard' is often a response to inappropriate property development in a local neighbourhood that is considered a

good idea if it occurred elsewhere. Due to the gap between undergraduate curriculum and the reality of ICU patient care, SRNs believed it would be better for NGN Transition to Professional Practice programme placement to be in a less critical clinical area:

Because they're fresh from the uni. They haven't got the skills. (Camilla, Line. 73–74, p.91)

I definitely think they should find out how a hospital works and have a bit of respect for the patient that you've just discharged to that unit

[Kath, Line. 448–449] (Whittam, 2018, p. 94).

The SRNs perceived that gaining RN experience before entering ICU would embed basic nursing skills and provide a broader knowledge of the patient's entire admission journey, including the integration of ICU within the wider hospital.

As SRNs deemed the ICU to be a stressful and overwhelming location, it was thought to be a sub-optimal clinical area for NGNs to acquire confidence in their newly obtained ICU skills and knowledge and clinical decision-making. The SRNs stated NGNs were learning to set priorities, manage their time, still developing aspects of basic care and task focused:

... like washing the patient say, or, you know, making sure they have their teeth cleaned. Just those little things that can make a difference to the [ICU] patient.

Because here even—you still have to do all that

[Lisa, Line. 184–186] (Whittam, 2018, p. 91).

The SRNs argued that first rotation NGNs should not be allocated to ICU due to the gulf between the NGNs' basic skill set and the interventions required by critically ill patients.

I don't know if we should ever have first rotation new grads in ICU ... I think that's dangerous

[Camilla, Line. 71–72] (Whittam, 2018, p. 92).

It was perceived by SRNs that there were insufficient resources, time and supernumerary staff to maintain sufficient first rotation NGN supervision and provide continuing, in situ clinical education, to decrease the possibility of errors in patient care:

It's—it's overwhelming. I think some of them are just paralysed with fear and take forever to learn things

[Camilla, Line. 223–224] (Whittam, 2018, p. 92).

Demonstrating their commitment to NGNs' clinical/professional development, SRNs highlighted the importance of NGNs obtaining clinical experience in acute hospital wards. The SRNs considered essential nursing skills, such as patient assessment, patient hygiene, ability to prioritise and time management, would enhance NGNs' nursing practice in ICU:

... basic things like that that you might've picked up on a more—a less critical area like a ward, is a really good thing to have under your belt, in terms of confidence and patient safety

[Kath, Line. 391–393] (Whittam, 2018, p. 93).

Participants recommended that gaining experience in acute hospital wards facilitated NGNs to embed patient assessment and surveillance skills without over-reliance on invasive technology inherent in ICU nursing work:

You have to actually use your stethoscope yourself. You have to count their [the patient's] respirations. You have to feel their pulse. You have to um, do a manual blood pressure. You have to talk to them. You have to reassure the relatives and then you have to build a case and take it to the doctors and explain it in a clinical and professional way. I think that's a really important skill to have

[Kath, Line. 435–439] (Whittam, 2018, pp. 93–94).

Additionally, SRNs wanted to be able to mitigate the emotional impact on NGNs from the ICU environment:

Yeah. It's horrible to think that anyone went home and, upset—like after a shift ... you want everyone to feel okay with their work. You, kind of just have to accept that new grads are going to feel really stressed ... I hate to see them like staying late or feeling really stressed, and they do. They tell you that they feel so stressed

[Sarah, Line. 318–322] (Whittam, 2018, p. 92).

The SRNs proposed that NGNs gaining experience in another clinical area's 'backyard' before working in ICU would reduce feelings of stress in both groups. Yet, variance existed among participants; some considered ICU an unsuitable environment for any RN without 12 months of acute nursing experience, while others supposed an ICU placement more suitable as a second rotation, provided ample resources were available to enable continuous NGN support and supervision:

... get all second rotation rather than first rotation—I think that would be ideal

[Camilla, Line. 196] (Whittam, 2018, p. 93).

The SRNs demonstrated care towards NGNs, using empathy and concern for influencing NGN development in a clinically and emotionally safe way.

Stories from SRNs working with NGNs in ICU told of experiences that balanced pressure from the critical nature of the *place* and *sociality* commonplace of this inquiry, framing the development of expertise within the ICU nursing team. The stories were bounded by

their temporal nature and the *temporality* of the 6-month ICU placement at the beginning of a nursing career.

8 | DISCUSSION

As one part of the solution to the nursing shortage, NGNs are recruited to work in ICUs. This inquiry sought to explore SRNs' experiences working with NGNs in ICU. Knowing the critical and pressured environment of ICU and holding the belief that NGNs are not adequately prepared to nurse safely in this context, SRNs suggest that NGNs gain initial experience elsewhere. The SRNs felt responsible for patient safety yet perceive time and resource impediments to their surveillance of NGNs. In this context, SRNs rely on NGNs' initiative to seek help; however, SRNs suspect that NGNs' reluctance to ask questions may indicate they are trying to conceal deficits in their knowledge. Despite the perceived burden of NGNs in ICU, SRNs find their contribution to NGN development rewarding.

The ICU RN shortage has impelled ICUs to employ NGNs. It is reported 23.3% of NGNs are employed in critical care areas in the USA after registration (National Council of State Boards of Nursing [NCSBN], 2018). In this inquiry and other reports (Ballem & MacIntosh, 2014; Baumberger-Henry, 2012; O'Kane, 2012), SRNs raised concerns regarding the inadequate preparation of NGNs, limiting their capacity to nurse high-acuity patients. Lack of ward experience may hinder NGNs' development in ICU (O'Kane, 2012). Experience in clinical areas with limited technology provides NGNs with opportunities to embed comprehensive nursing assessment skills: 'accountability, responsibility and the ability to prioritise' (O'Kane, 2012, p. 49). The finding of perceived NGN unpreparedness aligns with other studies recommending that NGNs should gain experience in ICU as a student (O'Kane, 2012). As higher acuity patients are increasingly managed in generalist wards, graduating nurses should have exposure to the knowledge and skill set necessary to manage critical patients wherever they are located (Williams & Palmer, 2014). However, providing ICU clinical placements for nursing students increases pressure on SRNs to deliver quality clinical education and supervision (Mould et al., 2011). The placement of nursing students in ICU raised SRN concerns which mirror those described in this inquiry when reflecting on the NGN experience: staff shortages and balancing time for patient care with teaching (Carlson et al., 2010).

In this inquiry, SRNs recognised their vital role in the surveillance of inexperienced NGNs, 'carrying' them by intervening when required to rescue ICU patients from harm. However, SRNs perceived that increased patient acuity and lean staffing levels, both in numbers and skill mix, impede capacity to provide adequate surveillance and assistance. Twigg et al., (2013) emphasise patient safety 'can only be realised if there are sufficient nurses to provide appropriate care' (p. 544). High nurse workloads in ICU result in poorer patient outcomes, increased mortality and hospital-acquired complications (Gurses, Carayon, & Wall, 2009). Healthier work environments are associated with better quality of care patient outcomes and significantly lower

odds of job dissatisfaction and nurse burnout (Aiken et al., 2011). Staffing issues are complex in ICU. The AACN (2016) recommend basing RN staffing levels on aggregate patient acuity rather than inflexible nurse-to-patient ratios and total patient numbers. Static ratios disregard variability in patient acuity and needs, the status of the dynamic ICU work environment and individual nurse capability. We found that SRNs noted the lack of 'ACCESS' RNs in the inquiry place. Adequate staffing resources to manage SRN workload, maximise patient safety and optimise ICU bed utility may be achieved by rostering the recommended 'ACCESS' RNs (Chamberlain et al., 2018, p. 297), providing NGNs with better access to, including consultation and education time with, SRNs. Despite this, policies for employing additional SRNs are compromised by both sustained ICU nursing workforce shortages and imposed hospital budget austerity measures (Willis et al., 2017).

Communication errors are one of the main causes of adverse events in hospitals (Slade et al., 2019). In this inquiry, SRNs speculated that sometimes NGNs concealed their clinical experience and knowledge levels and seemed reluctant to seek guidance because of embarrassment, fear and the presence of patients' families. This finding is supported by Saghaei et al., (2012) who reported that ICU NGNs tried to conceal their lack of experience from patients and their families, supposing their novice status could influence the degree of confidence in their capability to deliver patient care. The NGNs' goals of independence inhibited them from asking questions and seeking SRN's advice (Baumberger-Henry, 2012). In view of the demanding workload in ICU and perceived NGN unpreparedness, SRNs noted the significance of NGNs asking questions and requesting guidance, observing there may be increased risk of patient error when NGNs failed to do so. A lack of NGN questions reduced SRN capacity to amend workloads to provide necessary NGN surveillance, support and education.

Despite increased workloads, and associated stress and pressure working with NGNs, SRNs spoke of their determination to be approachable and supportive. Recognising that initial experiences in nursing could impact NGNs' future careers, SRNs actively cared for NGNs, intending to make NGNs' ICU placement a positive experience. However, our findings contrast with studies describing unsupportive SRN behaviours (Baumberger-Henry, 2012; Parker et al., 2014), such as ignoring or remaining silent when witnessing non-constructive interactions with NGNs. An explanation for the divergence in findings may be variances in workplace culture, nursing leadership capability and resource availability between research sites.

High nursing workloads and the resulting stress may jeopardise patient safety due to increased burn out rates, the negative effects on communication, reduced staff motivation and decreased job satisfaction (Dilig-Ruiz et al., 2018; Hoonakker et al., 2011; Moss et al., 2016). In this inquiry, and other studies (Ballem & MacIntosh, 2014; Baumberger-Henry, 2012; O'Kane, 2012), SRNs report feeling stressed and overwhelmed working alongside NGNs in the ICU. Particular to this inquiry, SRNs' revelations of experiences of irritability, tiredness and labile emotions may be indicators of burnout

syndrome, which is linked to increased intention to leave, resignation and absenteeism rates (Aiken et al., 2012). Other factors contributing to burnout rates, for example, patient mortality and ICU length of stay (Merlani et al., 2011) and the presence of conflict (Merlani et al., 2011) are multifaceted issues and challenging to address. Avoidance of burnout syndrome could be considered an important quality improvement target. Hospitals need to generate workplace cultures and models of operation that appreciate SRNs' workload, via recognition, appropriate staffing and leadership support (Olds et al., 2017). We found that SRNs sought managerial recognition regarding the work involved and the parallel pressures of supporting NGNs while ensuring ICU patient safety. Performance evaluations are an opportune time for ICU nurse leaders to recognise the role of SRNs and the concomitant increased workload when they are educating and supervising NGNs (Ballem & MacIntosh, 2014). Frequent informal feedback is directly associated with lower turnover (Van Waeyenberg et al., 2015). We recommend that SRNs' workloads be reviewed regularly and potentially modified as targets for improvement interventions. As sufficient staffing and personnel resources are positively associated with ICU RNs' job satisfaction (Dilig-Ruiz et al., 2018) and, conversely, job stress, burnout and emotional exhaustion is negatively associated (Dilig-Ruiz et al., 2018), this is an area for further research. However, high stress factors inherent to ICU RNs roles, such as managing life-threatening illnesses and supporting families experiencing extreme loss and stress, may not be amenable to staffing approaches alone.

Networking and educational opportunities, as protected time away from clinical duties, improve job satisfaction and create positive experiences for those in similar roles (Goss, 2015). We recommend that nurse managers and organisational leaders enable SRNs access to education and communities of practice that specifically support the domains of their role. An emphasis on adult learning theory, evaluation and performance appraisal approaches, methods for providing and accepting constructive feedback, advanced communication and negotiation skills would be valuable in providing the skill set SRNs need to provide NGN clinical supervision (Powers et al., 2019). However, the complexity of ICU environments, ICU RN turnover and shortages, and financial constraints curtails opportunities for planned or spontaneous professional development (Alshahrani, 2020). Nonetheless, SRNs expressed the desire to be acknowledged and the need to be supported in their contribution to the development of NGNs.

One measurement of success of the Transition to Professional Practice programme is when NGNs remain working in ICU after completion. Intention to stay in ICU is related to the NGNs' not being required to practice in clinical conditions outside their capacity. Two significant and independent predictors of NGNs' intention to remain in critical care areas are 'high satisfaction with unit orientation' and 'high satisfaction with clinical supervision' (Hussein et al., 2019, p. 400). This inquiry place induction programme offers substantially less support time to NGNs than other ICU orientation and transition programmes presented in the literature (Bortolotto, 2015; Friedman et al., 2011), which document

12–26 weeks duration (Bortolotto, 2015; Friedman et al., 2011). Additionally, NGN ICU supernumerary periods differ in the literature, ranging from periods of 8-weeks (O'Kane, 2012) to an entire 3-month rotation (Missen et al., 2016). We recommend well-resourced NGN ICU orientation and supernumerary programmes that deliver sustained clinical supervision, graded entry to ICU nursing responsibility, didactic education and realistic workloads. There is some evidence that ICU induction programmes are cost-effective in terms of reduced turnover and increased RN retention (Friedman et al., 2011). Decreased nursing leadership appreciation of the benefits of well-resourced and extended NGN ICU orientation and supernumerary programmes, as well as fiscal limitations, may affect both SRNs' and NGNs' workloads; compromising the capability to provide safe and effective patient care. Collaboration with academic nursing faculty may help nursing educators and managers develop and evaluate evidence-based programmes, with the dual function of increasing support for SRNs and NGNs while expanding undergraduate curriculum content to include current understanding of complex clinical areas such as ICU (Mårtensson et al., 2013). Further research is needed on NGN ICU Transition to Professional Practice programmes to emphasise both SRN and NGN experiences, establish the financial and educational outcomes, and consider constraints when comparing ICUs (due to variances in ICU nursing staffing, financial and health organisation models, and patient cohorts).

We found that SRNs nurture NGNs, influenced by their own early nursing experience (*temporality*), their commitment to the nursing profession (*sociality*) and the satisfaction of contributing to NGN professional growth in the ICU context (*place*).

8.1 | Limitations

Although the narrative inquiry *sociality* commonplace brings the relationship between the inquirer (SW) and SRN participants to the forefront, this is a potential limitation via perceptions of status and power impeding the inquirer's access to, and analysis of, SRNs' experience. However, reflexivity was practised in peer-debriefing research team meetings and documented in field notes throughout the inquiry.

The inquiry was conducted at a single site where a homogenous cohort of five participants self-selected; therefore, the experiences presented may not reflect those of SRNs in other ICUs and are not envisaged for generalisation. Narrative inquiry research texts are richly textured and represent the complex stories of experience lived and told by participants alongside the primary inquirer; honouring NI's ontology of experience. Enrolling a representative and larger sample of nurses to produce generalisable themes would have required a different methodology (Clandinin & Connelly, 2000).

To fulfil the requirements of this Higher Research Degree project, milestones had to be reached within pre-determined timeframes. This limited the capacity of the primary inquirer to hear and

be attentive to more than five participant stories regarding the phenomenon under inquiry.

The potential limitations are ameliorated, and trustworthiness enhanced, by presenting the SRNs' narrative accounts, allowing readers their own interpretations and application to other settings [*sociality*] (Appendix S2). The narrative accounts and threads presented in the findings highlight 'the temporal, unfolding, contextual nature of the threads rather than the certainty of the threads as fixed, frozen, or context (life) independent' (Clandinin, 2013, p.143).

9 | CONCLUSION

Although limiting ICU employment to RNs with acute nursing experience may not be possible in the current healthcare environment, there is an essential need to generate and maintain healthy ICU workplaces. Therefore, we recommend reducing SRNs' workloads when they are supporting NGNs and developing evidence-based, designed, evaluated and well-resourced NGN ICU orientation and supernumerary programmes. Furthermore, a combination of managerial recognition, access to academic collaboration and education that enhance SRNs' education capacity and staffing that reflects total patient acuity may better support both the SRNs and NGNs.

This inquiry adds to existing nursing knowledge by making visible the SRNs' ICU experience. 'Readers of this current inquiry are invited to immerse themselves in the three-dimensional space; opening up new possibilities for different stories of SRNs' experiences with NGNs in the ICU to be lived and told' (Whittam, 2018, p. 121).

10 | RELEVANCE TO CLINICAL PRACTICE

The findings of this inquiry have implications for policy decisions regarding NGN placement in ICU on a Transition to Professional Practice programme. The NGNs enter the high-acuity, unpredictable ICU environment with a recognised gap between their existing clinical competence and the nursing requirements of ICU patients. Regarding ICU patient safety and quality of care, SRNs serve an essential role by providing surveillance, supervision and support while NGNs are delivering nursing care. However, SRNs perceive that they do not have enough resources to adequately support NGNs develop their clinical competence in the ICU, which may have implications for the quality of patient care provided. Although there may be financial implications for ICUs and healthcare organisations, staffing decisions should be informed by the match between the total clinical competence of the ICU nursing team and the needs of ICU patients, to achieve best patient outcomes while sustaining healthy work environments. The clinical capacity of NGNs at the start of their career, SRNs' workloads and ICU nursing staffing levels, is important factors to consider when designing and evaluating NGNs ICU induction and supernumerary programmes. Through SRNs' stories of experience, this inquiry has provided an understanding of the ramifications

for SRNs as they work with, care for and guide the development of NGNs in ICU.

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CONFLICT OF INTEREST

The authors declare no conflict of interest in this work.

AUTHORSHIP

All authors have confirmed that all authors meet the ICMJE criteria for authorship credit (<http://www.icmje.org/recommendations/browse/roles-and-responsibilities/defining-the-role-of-authors-and-contributors.html>) as follows:

- Substantial contributions to the conception or design of the work; or the acquisition, analysis, or interpretation of data for the work; AND
- Drafting the work or revising it critically for important intellectual content; AND
- Final approval of the version to be published; AND
- Agreement to be accountable for all aspects of the work in ensuring that questions related to the accuracy or integrity of any part of the work are appropriately investigated and resolved.

DATA AVAILABILITY STATEMENT

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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SUPPORTING INFORMATION

Additional supporting information may be found online in the Supporting Information section.

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